Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

INFORMED CONSENT FORM

Adenoidectomy

Removal of the adenoids

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Additional procedures in the same session: ☐ Tonsillectomy ☐ Ventilation tube(s) ☐ None _____

Reason for surgery (completed by the doctor):
☐ Nasal blockage / snoring / sleep apnoea
☐ Glue ear / recurrent ear infections
☐ Recurrent adenoiditis / sinusitis _____

Other / comments: _____

1 Your condition and the aim of the procedure

The adenoids are lymphatic tissue, part of the body's immune system, located at the back of the nose above the roof of the mouth. They may enlarge, particularly in children aged 2–7, and usually shrink from adolescence onwards.

Enlarged adenoids can block the back of the nose, causing **mouth breathing, snoring, pauses in breathing during sleep, a runny nose, a nasal-sounding voice and frequent infections**. Because they can also block the opening of the Eustachian tube, which connects the ear to the back of the nose, they may lead to **fluid in the middle ear (glue ear), reduced hearing and repeated ear infections**. Long-term mouth breathing may also affect the development of the face and teeth.

The aim of adenoidectomy is to remove the enlarged adenoid tissue in order to make nasal breathing easier and to reduce ear, sinus and airway problems. Your doctor will explain separately why this operation has been recommended for you or your child.

2 How is the procedure performed?

The operation is performed under general anaesthesia in an operating theatre. It is done through the mouth; **there are no cuts or scars on the outside.**

1. **Preparation:** After the anaesthetic has been given, a special instrument is placed to keep the mouth open and the soft palate is gently lifted so that the back of the nose can be seen.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

2. **Removing the adenoids:** The adenoids are removed from the back of the nose using a curette (a special scraping instrument), an endoscope-guided microdebrider, coblation or diathermy. As the adenoids are a tissue rather than a separate organ, they cannot be removed "by the roots"; the aim is to safely remove the part causing the blockage.
3. **Controlling bleeding:** Pressure is applied to the area with a swab for a short time and, if needed, bleeding is stopped with diathermy. No packing is left at the back of the nose at the end of the operation.
4. **Additional procedures:** If planned, removal of the tonsils (tonsillectomy) or insertion of ventilation tubes into the eardrums is carried out under the same anaesthetic. Separate consent forms are completed for these procedures.
5. **Waking up:** After the operation the patient is closely monitored in the recovery room until fully awake.

If the doctor considers it necessary, the removed tissue is sent for pathological examination.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 20–30 minutes (excluding anaesthesia time)	Usually home the same day; one night's observation if needed

3 Expected benefits and likelihood of success

- Easier nasal breathing, with less snoring and fewer pauses in breathing during sleep
- Better-quality sleep, and as a result improved daytime attention, behaviour and school performance
- Fewer episodes of middle-ear fluid and ear infections, helping hearing to improve
- Less nasal discharge, and fewer episodes of sinusitis and upper respiratory infections

In carefully selected patients, nasal blockage and snoring improve markedly in the great majority of cases. However, if the blockage has other causes as well — such as allergy, enlarged turbinates or a deviated septum — the symptoms may not resolve completely. Success for ear problems also depends on the development of the Eustachian tube; some children may need further treatment or ventilation tubes.

4 Other treatment options

- **Watchful waiting:** If symptoms are mild, it is possible to wait with regular check-ups, as the adenoids shrink with age.
- **Medicines:** Steroid nasal sprays can temporarily reduce adenoid swelling and blockage in some children; if there is an allergy, it is treated. The effect usually lasts only as long as the medicine is used.
- **Ventilation tubes alone for ear problems:** In some cases, inserting tubes into the eardrums without removing the adenoids may be considered.
- **Other options for sleep apnoea:** In very rare cases where surgery is not suitable, a positive airway pressure device (CPAP) may be used during sleep.

5 What happens if the procedure is not performed?

If the operation is not carried out, the adenoids may shrink by themselves during adolescence. Until then, however, mouth breathing, snoring, poor sleep, recurrent infections and hearing loss due to fluid in the ear may continue. Long-standing and severe sleep-disordered breathing can, in children, lead to slower growth and development, attention and behaviour problems and, rarely, strain on the heart and lungs. Middle-ear fluid that persists for a long time can affect speech development and cause permanent changes in the eardrum.

You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

6 Preparing for the procedure

- Blood tests (particularly bleeding and clotting tests) and any other necessary investigations are carried out before the operation, and you will be assessed by the anaesthetist.
- Please tell us if there is a family history of easy bleeding, bruising or a bleeding disorder.
- No solid food or milk should be given for **6 hours** before the operation; for clear fluids, follow the time given by the anaesthetist.
- Please let us know if there has been a fever, diarrhoea, cough or flu in the 2–3 weeks before the operation; it may be necessary to postpone it.
- If there is a loose baby tooth, tell your doctor, as it could come out during the anaesthetic.
- We recommend preparing your child for the operation using age-appropriate language that does not frighten them.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

Day of surgery	After waking up there may be a mild sore throat, blood-stained mucus from the nose and mouth breathing. Vomiting or a slight temperature may occur. Once any nausea has passed, water is given first, then lukewarm, soft foods. Most patients go home the same day.
First 3–5 days	Nasal blockage and snoring may still be present because of swelling; there may be bad breath, a slight temperature, earache and neck pain. Pain is usually mild and is controlled with the painkillers your doctor recommends. Drinking plenty of fluids is important.
Week 1	Most children can return to school or nursery within 3–7 days, and adults to work. Avoid crowded places and people with infections during the first week.
Weeks 2–4	As the swelling settles, nasal breathing improves noticeably. Any temporary nasal-sounding voice usually settles during this period. Swimming and strenuous sport are generally allowed after 2 weeks.
Months 1–3	A follow-up examination is carried out; if the operation was done for ear problems, a hearing test and tympanometry are used to check the ears.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Mild sore throat and discomfort when swallowing

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

- Blood-stained mucus from the nose and nasal blockage for a few days
- Bad breath (from the healing surface; settles within 1–2 weeks)
- Earache (referred pain from shared nerves; it does not mean there is a problem in the ear)
- Slight temperature (first 1–2 days), nausea and vomiting from the anaesthetic

Common conditions (most are temporary or resolve with treatment)

- **Temporary nasal-sounding voice** — It takes time for the palate to adapt after the adenoids are removed; air may escape through the nose when speaking for a few days to a few weeks, and occasionally fluids may come back through the nose. This usually settles by itself within 1–2 months.
- **Neck pain and stiffness** — Because of the operating position and healing at the back of the nose, moving the neck may be painful in the first few days; this usually settles within a few days.
- **Symptoms not resolving completely** — If there are additional causes such as allergy, enlarged turbinates or a deviated septum, nasal blockage may partly continue; further treatment may be needed.

Uncommon conditions

- **Bleeding** — Significant bleeding after the operation occurs in **fewer than 1 in 100 patients (about 0.5–1 in 100)**. It most often occurs in the first 24 hours. If the bleeding cannot be stopped, **a return to the operating theatre under general anaesthesia to stop it** may be necessary. A blood transfusion is very rarely needed.
- **Dental damage** — Pressure from the instrument used to keep the mouth open may chip or dislodge a tooth, particularly one that is already loose or decayed.
- **Injury or swelling of the lips, tongue or palate** — Small sores or temporary swelling may occur from contact with instruments; they heal within a few days.
- **Infection** — May show as a rising temperature, worsening bad breath and increasing pain; it usually improves with antibiotics.
- **Regrowth of the adenoids** — Adenoid tissue can grow back (in about **1–5 in 100** patients), especially in those operated on at a very young age and those with allergies. If symptoms return, a repeat operation may rarely be needed.

Rare or very rare, but potentially serious conditions

- **Permanent nasal speech / fluids coming back through the nose (velopharyngeal insufficiency)** — Occurs in about **1 in 1,500–3,000** patients; the risk is higher in children with a hidden cleft of the palate (submucous cleft). It usually improves with speech therapy; rarely, a corrective operation may be needed. This is why the palate is examined carefully before surgery.
- **Slipping of the neck vertebrae (Grisel syndrome)** — A **very rare** condition in which inflammation at the back of the nose affects the ligaments of the upper neck vertebrae. It shows as neck pain that starts in the days after surgery and gets steadily worse, inability to turn the head, and the head being held tilted to one side. If recognised early it is treated with medicines and a neck collar; if diagnosis is delayed, longer treatment or, rarely, surgery may be needed and permanent restriction of neck movement may remain. Steadily worsening neck pain must therefore always be reported.
- **Scarring or narrowing at the back of the nose** — Very rarely, especially when extensive tissue is removed together with a tonsillectomy, narrowing may develop at the back of the nose during healing; this may cause nasal blockage and require further treatment.
- **Severe bleeding and its consequences** — Extremely rarely, in the presence of an abnormally placed blood vessel or an unrecognised clotting disorder, severe bleeding may occur. In this case emergency surgery, blood transfusion and **intensive care** treatment may be needed; blood entering the airway can cause breathing problems.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.
- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Fresh, red blood** from the mouth or nose, spitting or vomiting blood (any bleeding beyond a few spots should be reported)
- Frequent swallowing (in children an important sign that blood is being swallowed), paleness, weakness
- **Fever above 38.5 °C**, or a fever that does not settle with painkillers
- Not drinking at all, no urine for 6–8 hours, unusual drowsiness (signs of dehydration)
- Steadily worsening **neck pain**, inability to turn the head or the head held tilted to one side
- Difficulty breathing or marked pauses in breathing during sleep

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- For pain, use only **the painkillers recommended by your doctor**; aspirin and aspirin-containing medicines must not be used, as they may increase bleeding.
- In the first few days, lukewarm or cool soft foods and **plenty of fluids** are recommended; avoid very hot, hard and spicy food.
- Do not blow the nose forcefully for the first week.
- If a return flight abroad is planned in the first week after surgery, discuss with your doctor the earliest time it is safe to fly; because of the risk of bleeding, it is advisable to stay within reach of your doctor for the first few days.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.
- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

14 Patient declaration

- ☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.
- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ It has been explained to me that in the first few days after surgery, particularly for my child, I must watch for signs of bleeding (frequent swallowing, spitting or vomiting blood) and contact the doctor.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

Op. Dr. Murat Ayata Private Clinic
Karacaibrahim Mah. Dereüstü Sk., Merkez, Kırklareli, Türkiye
Tel: +90 288 502 50 20 · WhatsApp: +90 555 333 21 20
info@muratayata.com · www.muratayata.com

FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.